

Discharge Summary USE BLACK INK ONLY

Patient Details		Admission and GP Details	
Surname Bradley		Discharging Consultant Dr E. Marchetti	
Forename Susan	M / F/ (Female)	Discharging Speciality/ Department Respiratory Medicine	
Date of Birth 27/09/1951		Method of Admission Emergency - GP referral	
NHS/ Hosp No. 512 897 6634		Date of Admission 20/07/2026	
Address 9 Meadow Rise,		Date of Discharge 25/07/2026	
Farnborough, GU14 8XZ		G.P. Details Dr T. Nguyen,	
Tel No. 01252 611 7789		Farnborough Medical Practice	
Diagnosis at Discharge		Operations and Procedures	
Infective exacerbation of COPD. PMH: hypertension, osteoarthritis, GORD.		None.	
Reason for Admission and Presenting Complaint(s)			
4-day history of increased breathlessness, increased sputum volume/purulence (green) and wheeze on a background of known moderate COPD. No fevers, no chest pain.			
Clinical Narrative			
74F, ex-smoker (30 pack-years, quit 2015) with known COPD (FEV1 58% predicted) presented with a typical infective exacerbation. Treated with nebulised salbutamol/ipratropium, oral prednisolone 30mg for 5 days and oral doxycycline for 5 days per local antimicrobial guidelines. Oxygen saturations maintained on air throughout; ABG showed no hypercapnia. Symptoms improved over 48 hours and she was stepped down from nebulisers to inhalers. Physiotherapy input given for sputum clearance, and inhaler technique reviewed and corrected by the respiratory nurse specialist.			
Relevant Investigations and Results			
WCC 12.1, CRP 62. CXR: hyperinflated, no consolidation. ABG: pH7.38, pCO2 5.4. Sputum: H. influenzae.			
Discharge Destination			
Home, lives alone; independent with ADLs, supported by neighbour and daughter (weekly visits).			
Relevant legal Information (e.g. was an independent Mental Capacity Act Advocate required)			
No safeguarding or Mental Capacity Act concerns; full capacity.			
Information given to patient and/or authorised representative (including e.g. see GP in 2 weeks)			
Advised on completing steroid/antibiotic course; inhaler technique corrected and demonstrated with spacer. Counselling on early recognition of exacerbations and smoking abstinence. Advised to contact GP/111 if breathless, feverish or coughing blood; call 999 if severe.			
Physical Ability & Cognitive Function :		On Admission	At Discharge
Physical	Breathless	Improved; baseline exercise tolerance (~100m)	
Cognitive	Normal	Normal	
Other	Wheeze	Chest clear, no accessory muscle use	
Advice, recommendations and future plans (including results awaited and outstanding investigations)			
G.P. Actions (Date) Review in 2 weeks; ensure pulmonary rehab referral has been actioned.			
For GP: review response to treatment in 2 weeks - 2nd exacerbation this year, consider stepping up inhaled therapy (e.g. triple therapy) if frequent exacerbator; check vaccination status; encourage pulmonary rehab attendance. Counselling given as above re inhaler technique, COPD self-management and completing treatment course.			
Strategies for potential problems			
COPD rescue pack discussed for future exacerbations per action plan; daughter aware.			

